Treatment scope · NCCP 00215 Lung context only. Confirm whether the platinum pathway or EGFR/erlotinib pathway applies. This guide does not supply the partner dose or replace its safety assessment.

NCCP 00215 · VERSION 7

Bevacizumab 15 mg/kg · lung treatment component

Bevacizumab 15 mg/kg is infused on Day 1 every 21 days. With platinum chemotherapy, the combined phase lasts up to six cycles, then bevacizumab may continue alone. With erlotinib, the combination continues while helpful and tolerated. Progression or unacceptable toxicity can stop either plan earlier; no fixed overall lung-treatment duration is promised.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and checks

These are relative planning stages, not booked appointments or equal time intervals. Extra blood-pressure, urine, heart or anticoagulation checks depend on individual risk and results.

Check	Before starting	First infusion	Later cycles	Between visits	Pause or change
Blood count / kidney / liver	Baseline tests	Review results	Before each cycle	Extra tests if needed	Review abnormalities and partner treatment
Blood pressure	Measure and control BP	Before and after treatment	Before and after each treatment	More often if raised or symptomatic	Review control before restarting
Urine protein	Baseline urine test	Review urine result	Before each cycle	24-hour collection if indicated	Recheck protein before agreed restart
Heart / anticoagulation	Heart history; ECHO if risk; INR if indicated	Review symptoms and medicines	Warfarin INR before cycles once stable	INR weekly until stable; report symptoms	Individual cardiac and anticoagulant review

THE WHOLE TREATMENT JOURNEY

Bevacizumab 15 mg/kg · lung treatment component: treatment and checks

First-line non-squamous NSCLC · platinum or EGFR/erlotinib context

Bevacizumab 15 mg/kg is infused on Day 1 every 21 days. With platinum chemotherapy, the combined phase lasts up to six cycles, then bevacizumab may continue alone. With erlotinib, the combination continues while helpful and tolerated. Progression or unacceptable toxicity can stop either plan earlier; no fixed overall lung-treatment duration is promised.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First infusion	Later cycles	Between visits	Pause or change
Bevacizumab treatment NCCP 00215	Confirm platinum or EGFR/ erlotinib pathway	Day 1 bevacizumab after checks	Every 21 days; review selected partner	Partner has separate schedule	Consultant review before restart or phase change
Platinum or erlotinib phase NCCP 00215	Select one source pathway	Begin separately assessed partner	Platinum: up to six combined cycles	Then bevacizumab alone if appropriate; erlotinib has no automatic switch	Review benefit, toxicity and complete plan
Blood count / kidney / liver NCCP 00215	Baseline tests	Review results	Before each cycle	Extra tests if needed	Review abnormalities and partner treatment
Blood pressure NCCP 00215	Measure and control BP	Before and after treatment	Before and after each treatment	More often if raised or symptomatic	Review control before restarting
Urine protein NCCP 00215	Baseline urine test	Review urine result	Before each cycle	24-hour collection if indicated	Recheck protein before agreed restart
Heart / anticoagulation NCCP 00215	Heart history; ECHO if risk; INR if indicated	Review symptoms and medicines	Warfarin INR before cycles once stable	INR weekly until stable; report symptoms	Individual cardiac and anticoagulant review

What each check is for

1 Confirm treatment scope

Before starting and whenever the partner changes

NCCP 00215f: first-line unresectable advanced, recurrent or metastatic non-squamous NSCLC with platinum-based chemotherapy. NCCP 00215g: the corresponding first-line EGFR-activating-mutation setting with erlotinib. This is the bevacizumab component; every partner needs its own prescription, assessment and consent.

Ask / act: Confirm indication, partner, phase and separate assessment.

Sources: NCCP 00215

2 Blood pressure and proteinuria

Baseline, each cycle and extra checks when indicated

These effects can be silent, so attend planned tests even when well.

Ask / act: Agree who books, reviews and acts on each test.

Sources: NCCP 00215

3 Procedures and urgent symptoms

Throughout treatment and treatment breaks

Seek urgent help for sudden severe abdominal pain, significant bleeding or coughing blood, chest pain, breathlessness, a swollen painful leg, a severe headache, confusion, seizures, a change in vision, or a rapidly painful/red spreading wound. Follow the chemotherapy emergency plan for fever or feeling acutely unwell.

Ask / act: Contact the team before procedures or for urgent symptoms.

Sources: NCCP 00215 · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.